ATRIA

HEALTHCARE APPOINTMENTS

HACKATHON PITCH

From the waiting room to the pitch

The three pain points we are building the pitch on, the evidence behind each, the Atria screen that answers it, and a five-minute running order.

DOCUMENT	REVISION	DATE	STATUS
ATR-PIT-001	Rev A	13 November 2026	Issued for review

PROJECT	TEAM	DEMO DATA	LENGTH
DIGISOL Internship 2026	Bryan, Thanksgiving	Synthetic preview	5 minutes

1 The flow map

Read left to right. Every row ends in something you can show on screen. Anything we only specified is labelled as such, on the slide and out loud.

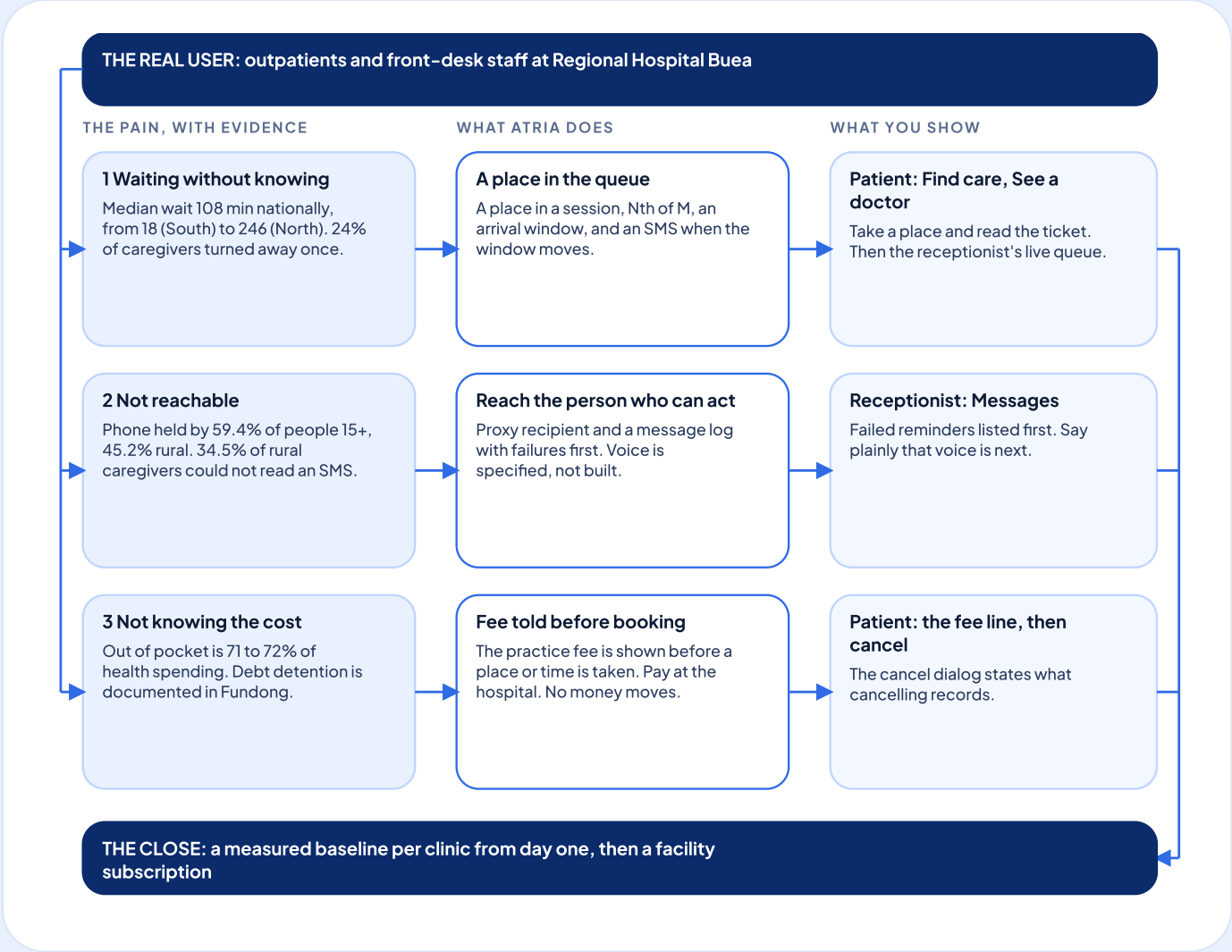


Figure 1. Pitch flow: the real user, three pain points, the Atria answer to each, and what to show.

The one-sentence pitch

At a district hospital in Cameroon, an outpatient does not know how long the wait will be, what the visit will cost, or whether the message sent to them is one they can use. Atria gives them a place in the queue, a fee before they come, and a reminder that reaches the right person.

2 Running order

Five-minute running order

TIME	BEAT	WHAT HAPPENS	WHERE	WATCH OUT
0:00	Hook	Say the user group first, then one number: median wait 108 minutes, from 18 to 246 by region.	Slide only	Cited: national immunisation survey, 1,230 caregivers, 265 facilities.
0:30	Act 1, waiting	Patient preview: Find care, See a doctor. Read the fee line. Pick a day and a part of the day, take a place. Open the ticket: arrival window, place Nth of M.	/__preview?as=patient	Do not reload the page, the preview resets on reload.
1:15	Act 1, the desk	Switch to the receptionist preview. Show the live queue and the day counts.	/__preview?as=receptionist	The desk sees the same ordered list. Walk-ins join it.
2:00	Act 2, reach	Receptionist, Messages: failed reminders sit at the top so the desk can phone. Name the proxy recipient. Then say: voice fallback is specified and next.	/__preview?as=receptionist	Do not demo voice or local-language SMS as working.
3:00	Act 3, cost	Back to the patient. Show the fee on the clinician page. Give up the place and show the cancel dialog stating what cancelling records. State: nothing is charged.	/__preview?as=patient	Cancelled ticket now says no place and no need to come.
4:00	Close	One slide: a measured baseline per clinic from day one, then a facility subscription with a message bundle. Ask for a pilot at the Regional Hospital Buea.	Slide only	No savings figure and no no-show percentage.

Before you go on stage

Start the web dev server, open the preview once, and rehearse without reloading. The preview data is in memory and resets on reload, so a refresh mid-demo returns you to the start. Run the demo on the same laptop you will present from.

3 What to say aloud, and what not to

Numbers safe to say on stage

PAIN	NUMBER TO SAY	SOURCE	CAVEAT TO KNOW
Waiting	Median 108 minutes, 18 in the South to 246 in the North; 24% turned away once	National immunisation survey, Vaccines 2024	Satisfaction was still 87%, so do not lean on satisfaction scores.
Waiting, local	Mean 150 minutes from registration to consultation, Douala	Bita Fouda et al. 2024	Facility-level and 2020 data.
Reach	Phone held by 59.4% of people 15+, 45.2% in rural areas	INS household survey ECAM5, 2022	Ownership is not the same as reading SMS.
Reach, SMS	34.5% of rural caregivers could not read an SMS	Bigna et al. 2014, 301 caregivers	Paediatric HIV carers, 2013 data.
Cost	Out of pocket was 71 to 72% of current health spending	Bousmah 2021; WHO country story	Years differ: 2017 to 2019.
Cost, access	58% of injured patients used the 24-hour payment deferral, yet 12.3% still missed radiology	Dissak-Delon et al., BMJ Global Health 2025	Injury patients in four hospitals.

Do not say these

Any no-show reduction percentage. Any money saved. Any clinician rating or ranking. The 26.86% global no-show figure, which the primary paper does not contain. The 20 million Waspito user claim, which conflicts with the company's later figure. Reminder evidence in Cameroon is split: one positive trial after missed visits, one null TB trial, and a pooled African effect of odds ratio 1.36 in good-quality trials.

4 What is built, and what is not

What is true on the day

CAPABILITY	STATUS FOR THE DEMO
Queue place and arrival window	Built, shown live in the preview
Ticket after cancelling says no place	Fixed this session, shown live
Fee before booking, queue and specialist	Added this session, shown live
Receptionist and clinician consoles	Built, load with no console errors
Failed-first message log	Specified and has a screen, not walked through live
Real SMS delivery over MTN, Orange, Camtel	Not verified, sender registration open (OI-04)
Voice fallback and local-language reminders	Specified, not built, no voice provider (OI-05)
Real patient data	Not allowed before the Authority's authorisation (Law 2024/017)

5 Questions to expect

Question preparation

LIKELY QUESTION	ANSWER
Does an SMS reminder reduce missed visits in Cameroon?	The evidence is mixed, and we say so. We do not promise a percentage. We measure a baseline at the first clinic and report it.
Why not an app?	Mobile subscriptions are about 108 per 100 people but internet use is 41.9 to 46.3%. The queue works by SMS and at the desk for people who install nothing.
How do you make money?	A facility subscription, with a message bundle priced above carrier cost, and district or programme licences where funders already pay for follow-up. Patients pay nothing to book.
What does a message cost?	We do not have a verified rate yet. Aggregator prices vary about tenfold, so we are asking carriers for a contract price before we quote one.
What about patient data?	Law 2024/017 treats health data as sensitive. Atria runs on synthetic data only until authorisation, keeps intake answers apart from the appointment, and audits every staff read.
Who else does this?	Waspito, E-Sante Cameroun and others are mostly patient-facing telehealth. Atria is for the facility that runs a queue. We have not verified their pricing.
Does a visible queue not cause arguments about order?	The server assigns position from the request time. A client cannot set it, and staff reschedules need a reason and are audited.